

04. THE ENDODERM : INTRODUCTION & GENERALITIES 2

DURATION : 11:50

STUDY SHEET

COURSE SUMMARY

This video offers an in-depth exploration of the endoderm, a crucial tissue in embryonic development, highlighting its role in the digestive and endocrine systems. You will learn how boundary tissue and inner tissue interact, affecting overall body health. Key concepts such as the cranio-sacral-sternal system and the importance of glands like the thyroid and adrenals are discussed. Combining theory and practice, this lesson will allow you to better understand tissue dynamics and their impact on the body, while also providing you with tools to work on facial and alimentary tissues to deeply support the endocrine system.

THE ENDODERM: INTRODUCTION AND GENERALITIES

The **digestive system** is suspended from the base of the skull, which implies an essential movement between the **sacrum** and the **occiput**. This movement is vital for health, as excessive tension in this area can disrupt the **cranio-sacral** system.

Embryonic development is characterised by a global movement, where the turning plate manifests at the **sternum**. Thus, we have a **cranio-sterno-sacral** system, which also extends to **mandibular** development. This process includes phases of expansion and retraction, as well as phases of inspiration.

The **teeth** play a crucial role in influencing the mapping of the base of the skull, which houses, in the sphenoidal plane, a gland related to the **pineal gland**. This gland is essential for the hormonal process, highlighting our dependence on our glands: **pineal**, **pituitary**, **thyroid**, **adrenal**, and **gonadal glands**. The balance between the **thyroid** and **adrenal** systems is fundamental, as these systems are involved in adaptation.

The glandular system depends on **connective tissue**. To release an endocrine system, it is necessary to work on the **fascial** and **alimentary** tissue. It is important to note the chronology of tissue appearance. The digestive system, suspended from the base of the skull, can be imagined as a resonating bell.

We have a tissue that divides into a **boundary tissue** and an **internal tissue**. The boundary tissue is primordial, and it is in this tissue that we will study the **endoderm**. The endoderm is an **epithelium**, a boundary and exchange tissue. With the **mesoderm**, it contains primary

information, acting as a royal road.

It is essential to understand that in boundary tissue, there is never congestion, as congestions are always treated at the **mesodermal** level. Mesodermal tissue carries all **vascular** systems.

Endodermal tissue, as a boundary epithelium, is also responsible for the glandular system. All glands derive from this boundary tissue, whether it is the **thyroid**, the **lungs**, or the **liver**. These organs are expansions of the epithelial tube, representing an epithelial endoderm.

The hormonal system extends deep into the body and is highly dependent on the digestive and ectodermal systems. The internal tissue acts both as a motor and as resistance to growth, limiting tissue development. This process can be described as a **developmental algorithm**, where growth rates vary within tissues, creating points of support called **Hitchpons**.

These hinge points are essential for the general organisation of the body. The body knows what it needs to do, but it is crucial to support this process by returning to **ontogenetic** qualities. A **fulcrum** is a point of form creation, serving as a balance point during changes.

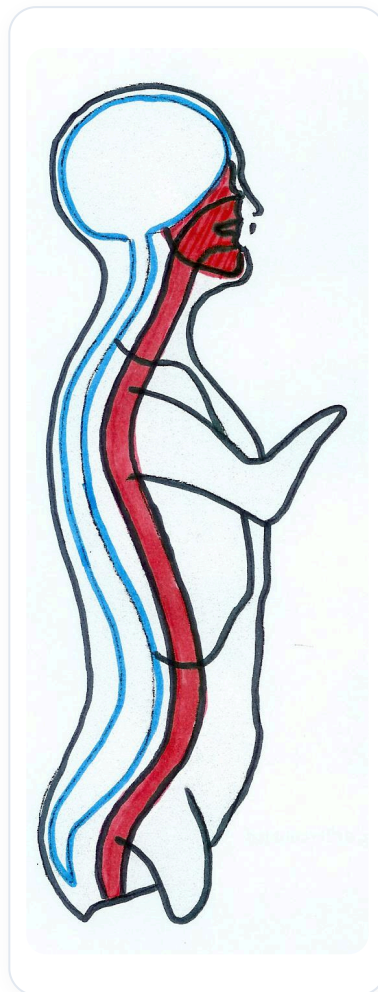


FIG. — Body plan

The vascular system, consisting of two primitive vessels, is integrated into the internal tissue, the **mesoderm**. This tissue contains vessels that develop early in the embryo. The reorganisation of the body's fluid system occurs as the **notochord** appears, bringing energy and acting as a motor,

while slowing down visual growth.

The **ectoderm**, on the other hand, develops rapidly, optimally nourished. The developing heart does not grow at the same rate as the ectoderm, thus becoming a global support point for development. The **sphenobasilar** symphysis and the heart constitute the first fulcrums of organisation.

The body's joints are sequential repetitions of this same process. For example, a kidney on a liver or a lung on a diaphragm represent phases of articulation. The capacity for movement is influenced by the presence or absence of arteries.

Initially, the embryo coils around its cardiac area, and brain development follows the vascular fields. This development is synchronised with the digestive system, and a phenomenon in one area can block a function in another.

The **navel** represents the initial contact during incarnation in the uterine earth. This first contact is crucial and always occurs in a specific pole, on an assimilative plane, where the embryo enters through an organised electrical field.

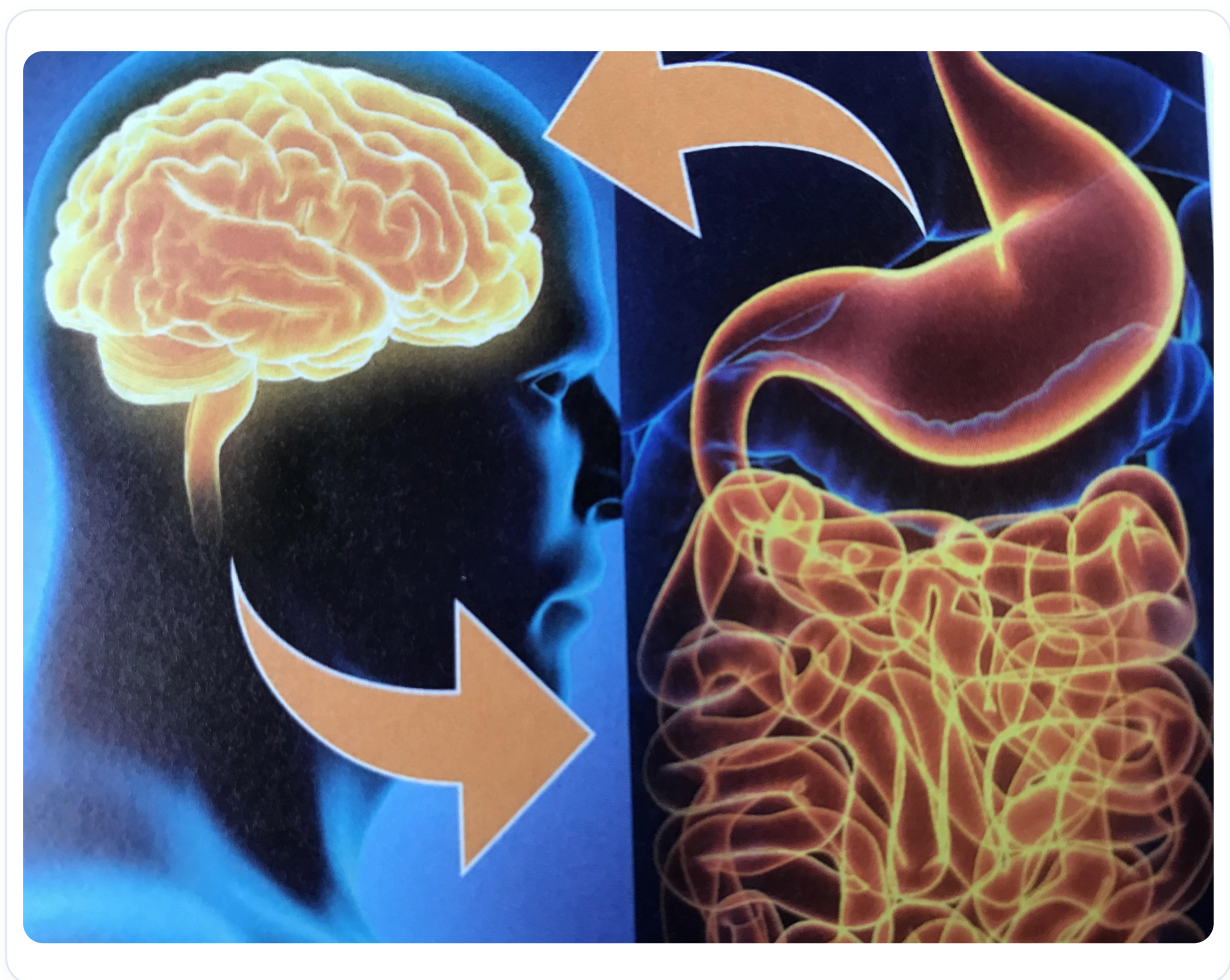


FIG. — Brain-gut axis

The umbilical cord, a vestige of the **vitelline vesicle** and the **embryonic pedicle**, symbolises this primitive phase of integration. It represents the link between incarnation and contact with the mother.

Finally, the intestine and the body symbolise the integration of the individual. The search for a personal symbol is essential to avoid dispersion and promote concentration and gathering.

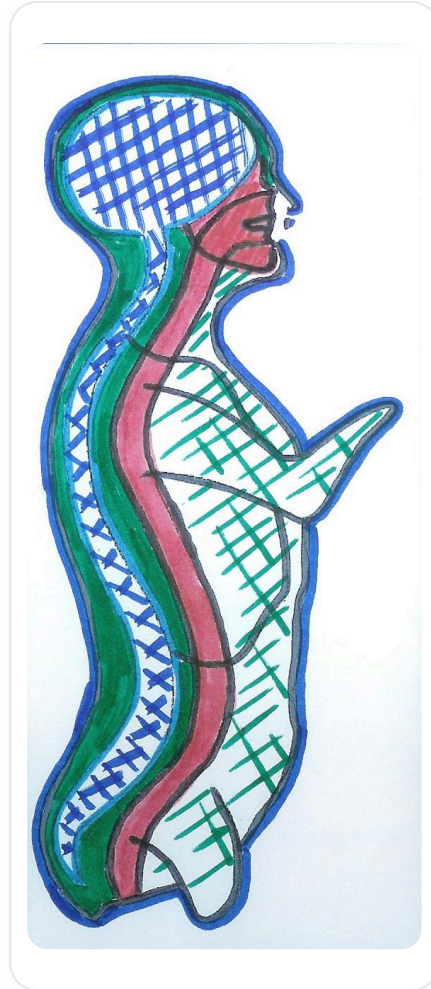


FIG. — Dermis and epidermis